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Ability to move between self-states and emotional experiencing and processing as predictors of symptomatic change

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Objectives. Contemporary relational theories consider clients' ability to move between multiple self-states and clients' ability to experience and process emotions to be two therapeutic processes inherently connected and fundamental to growth and change in psychotherapy. The current research aimed to empirically explore these theoretical assumptions by (1) examining the between- and within-client temporal association between clients' levels of ability to move between multiple self-states and clients' emotional experiencing and processing over the course of therapy, and by (2) testing whether this association would be more prominent for clients presenting good treatment outcomes.

Method. Based on treatment outcome measures, two contrasting groups were selected, nine successful and nine unsuccessful. Ninety session transcripts (five per client) were analysed using the Two-Person APES (TPA), an extension of the Assimilation of Problematic Experiences Scale (APES), for the measurement of the ability to move between self-states. Sessions were also analysed using the Experiencing Scale (EXP), for the measurement of emotional experiencing. For all predictors, we disaggregated within- and between-client effects.

Results. No association was found between EXP and TPA levels. However, at the between-client level, an interaction effect was found indicating a positive correlation between TPA and EXP only for the good-outcome cases, whereas a negative correlation between these variables was found for the poor-outcome cases.

Conclusions. The ability to move between multiple self-states and accessibility to emotional experiencing may be seen as complementary processes enhancing the effectiveness of therapy. However, a high level of one process combined with a low level of the other may be a sign of poor therapeutic outcome.

Practitioner points

- Clients' capacity to negotiate between conflicted self-states and their ability to experience and process emotions are two determinants working in a synergistic way to predict improvement in clients' symptoms.
- The presence of only one of these psychological processes without the other might be an indication of poor therapeutic outcome.

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- Therapists should aim to increase their clients' progress in both the movement between self-states and emotional experiencing.

The notion that one's identity comprises multiple internal entities has become prevalent within various psychological approaches (Bromberg, 2016; Gergen, 2009; Hermans & Dimaggio, 2004; Stiles, 2001). In object relations theory, for example, these entities represent introjects of significant others (Fairbairn, 1952; Klein, 1948). In Gestalt or process-experiential psychotherapy, they represent dialectical aspects of the self or representations of significant others (Greenberg, Rice, & Elliott, 1993; Perls, 1969), and in cognitive approaches, they encompass what have been called schemata (Beck, Epstein, & Harrison, 1983). Many of these approaches assume that promoting dialogue among these entities will generate therapeutic progress.

This idea of self-multiplicity has received significant empirical support from within the framework of the assimilation model (e.g., Basto, Stiles, Rijo, & Salgado, 2018; Stiles, 2002; Stiles & Angus, 2001). The assimilation model is an integrative model, aiming to account for change during therapies of different theoretical orientations. According to this model, the separate self-entities are defined as 'voices'. Each voice is composed of traces left by previous experiences, and the multiple voices interact with each other, forming a community of voices and an organized structure of the self (Stiles, 2011). Some voices, however, undermine the stability of the community as a whole. These problematic voices stem from experiences that are incongruous with the cohesive self (e.g., traumatic incidents or painful situations), and are therefore rejected, avoided, or dissociated from the community of voices. When problematic voices are disavowed, the self-community of voices tends to become rigid, important experiences are disregarded, and valuable resources are unavailable to the self, thus creating maladaptive functioning and distressing symptoms. Psychotherapy aims to promote the integration of problematic internal voices into the self-community, allowing for the renewal of dialogue between formerly detached voices (Stiles, 2002; Stiles *et al.*, 1990).

Stiles and colleagues developed the Assimilation of Problematic Experiences Scale (APES; Caro Gabalda & Stiles, 2009; Stiles *et al.*, 1991) as an empirical measure enabling the description of the various self-voices and their internal relations. The level of assimilation of voices exists along a developmental continuum: from denial of the problematic voice, through acceptance, to an integration of the excluded voice back into the community of voices (see Osatuke & Stiles, 2011). Numerous studies of this model have demonstrated that growth in therapy is associated with a progression towards higher assimilation levels, with the formerly dissociated voice(s) becoming increasingly integrated (for reviews, see Stiles, 2002, 2011).

Two-person APES

The Two-Person APES (TPA; HaCohen, Atzil-Slonim, Shapira, Bar-Kalifa, & Tuval-Mashiach, 2018; HaCohen, Atzil-Slonim, Tuval-Mashiach, Bar-Kalifa, & Fisher, 2017) represents an extension and modification of the assimilation model. In line with contemporary psychodynamic relational views, the TPA differs from the original APES in two main regards: First, the TPA emphasizes the ability to contain, rather than resolve, conflict between different inner voices; such containment is also assumed to be followed by symptomatic relief. Second, whereas the main focus of the APES is the client, the TPA views the relationship between client and therapist as the main 'vehicle' for change. For example, clients who refer to the treatment termination and say, 'I know how much you

and the treatment have helped me and what a different place I'm in today than when I started. And still, I also know that it will not be easy for me' would be rated as a level 4 (of 7) on the TPA, given that both the strong and independent voice and the voice of neediness exist and are marked both by pleasant emotions and bereavement. On the TPA, on the other hand, this client would be rated as a level 5 (of 5), as he/she is able to be in contact with both self-states and can contain the tension between the emotions accompanying them without the need to silence one of them. This process also occurs in the clear presence of the therapeutic relationship. Based on relational theory terminology, in the TPA model, the term *inner voices* has been replaced with the term *self-states* (Bromberg, 2003a, 2003b). The TPA conceptualizes six levels of movement quality between self-states, from dissociation to conflict to dialectic, and finally to a mutual co-creation of self-states (HaCohen *et al.*, 2017). As such, clients' ability to move between self-states regards the quality of this movement rather than the frequency of the movement.

A recent study comparing good- and poor-outcome psychodynamic treatments showed that good-outcome cases demonstrated a progression in TPA levels for both client and therapist, from dissociation to dialectic, over the course of the treatment. Poor-outcome cases showed no such progression (HaCohen et al., 2018, 2017).

Emotional experiencing and TPA

According to psychodynamic relational views, another core process that may lead to growth and change in psychotherapy is the client's emotional experiencing and processing. From these perspectives, psychotherapy is an opportunity to promote and maintain psychological health by using emotionally significant relationships to re-experience and process emotions that are too painful for an individual to manage alone (Castonguay & Hill, 2012; Fosha, 2001; Rachman, 2007).

Emotional experiencing, which is defined as the client's ability to approach, activate, and process emotions, is a highly studied process variable in psychotherapy research (Greenberg & Safran, 1984; Rachman, 1980). Numerous studies have shown that deep emotional experiencing during sessions is positively related to treatment outcomes across theoretical orientations and disorders (Greenberg, 2012; Thoma & McKay, 2015; Whelton, 2004). Emotional experiencing is most commonly measured using the Experiencing Scale (EXP; Kiesler, 1973; Klein, Mathieu-Coughlan, & Kiesler, 1986), which measures progression from detachment from emotions, to experiencing of emotions, to processing of emotions.

The progression in emotional experiencing and processing is in line with the theoretical models of both the APES (Stiles *et al.*, 1991) and its relational extension, the TPA (HaCohen *et al.*, 2017), but with several distinctions in accordance with the differences between these measures. Both APES and TPA models assume that clients are prone to be emotionally detached at earlier stages of the change process, as they defend themselves from threatening self-states, or voices. In more advanced stages, clients are expected to experience greater emotional arousal, as they begin to 'get in touch' with unprocessed emotions that accompany unwanted self-states, mostly negative and painful emotions that were previously dissociated. At these stages, the client may be flooded, making it hard to process emotions productively. As the treatment progresses, a recognition of emotions and their meaning becomes possible, and a growing understanding of previously detached emotions takes place (Dimaggio, 2006; HaCohen *et al.*, 2017; Stiles, Osatuke, Glick, & Mackay, 2004). However, unlike the APES, which strives for positive emotions or for a discharge and neutralization of the emotional load in the final stages of the model, the TPA emphasizes enlivened emotional experiencing (including

negative emotions), signifying the mutual co-creation of new self-states in its final stage. In addition, as opposed to the APES, the TPA takes into account the client–therapist interaction, and therefore, its relation to emotional processing can reveal a new understanding of the therapeutic process.

Though these theoretical models assume an association between clients' ability to move between self-states and emotional experiencing, only two studies have studied this association empirically. A recent case study of cognitive-behavioural therapy for depression using the APES examined the relation between assimilation level and emotional valence and salience (Basto, Pinheiro, Stiles, Rijo, & Salgado, 2017). Results showed that emotions worked as markers for the client's assimilation level, starting from repression of feelings, then gradually moving to negative feelings, and finally moving to positive feelings. These findings may be explained by the TPA model, which aims for heightened emotional experiencing rather than positive emotions or neutralization of emotions at later stages of the process. Another study (Rudkin, Llewelyn, Hardy, Stiles, & Barkham, 2007) examined the relation between the client's emotional experiencing, using the EXP (Klein *et al.*, 1986), and the client's assimilation process according to the APES. Results showed that clients who had higher levels of EXP also exhibited higher levels of assimilation.

Taken together, these studies suggest that progression in clients' ability to move between self-states is accompanied by progression in emotional experiencing. Still, more research is required to establish this link and to test whether the reported association also arises when examining the TPA. In addition, more research is needed to examine whether this joint progression is associated with treatment outcome. Previous studies were designed as case studies (as in Basto et al., 2018), or they analysed only one session for each client (as in Rudkin *et al.*, 2007). Case study designs only allow for the examination of *within-client* effects (i.e., whether *sessions* in which the client demonstrates higher levels of TPA are also characterized by greater emotional experiencing). In contrast, analysing one session for each client allows for the examination of *between-client* effects (i.e., whether *clients* who demonstrate higher levels of TPA will also demonstrate greater emotional experiencing).

The current study aimed to expand the inquiry of the relation between self-states movement ability and clients' emotional experiencing and processing via use of the TPA. The research examined a number of sessions from a sample of 18 clients, comparing good- and poor-outcome cases, to allow an examination of the research hypotheses at between- and within-levels simultaneously. We hypothesized that:

1. There would be a positive association between TPA levels and emotional experiencing levels over the course of therapy.
2. This association would be more prominent for clients presenting good outcomes.

Method

Participants and treatment

Clients

The participants were 18 adults (12 women) recruited from an existing pool at a large university outpatient clinic. Inclusion criteria included an initial score of between 17 and 40 on the Beck Depression Inventory-II (BDI-II), indicating mild to severe depression; receiving psychodynamic psychotherapy; and participating in a minimum of 10 therapy sessions.

Clients were over 18 years old (M_{age} = 42.66 years, *SD* = 13.71, age range 25–70 years). Eight of them were single or divorced, and 10 were married or in a long-term relationship. Half of them had at least a bachelor's degree, and 15 clients were fully or partially employed.

Diagnoses were based on the Axis I Diagnostic and Statistical Manual of Mental Disorders-IV (4th ed., text rev.; *DSM–IV–TR*; American Psychiatric Association [APA], 2000). The interview was conducted at intake by trained independent clinicians. Ten clients were diagnosed as suffering from depression and three from social or generalized anxiety. The remaining clients experienced adjustment difficulties but did not meet the clinical criteria for an Axis I diagnosis. According to pre-treatment assessments, the mean BDI-II score for the sample was 25.16 (*SD* = 6.55).

Therapists

The sample included 16 therapists (13 women) at different stages of clinical training (1–5 years of experience). Two therapists treated two clients each, and the others treated one client each. Each therapist received 1 hr of individual supervision and 4 hr of group supervision on a weekly basis. All therapy sessions were audiotaped for use in supervision. Supervisors were senior clinicians in psychodynamic psychotherapy. Individual and group supervision focused heavily on the review of audiotaped case material and technical interventions designed to facilitate the appropriate use of psychodynamic interventions. At the time of treatment, the therapists were unfamiliar with the TPA, the EXP, and the research hypotheses.

Treatment

Individual psychotherapy consisted of one weekly session of psychodynamic psychotherapy organized, aided, and informed by a blend of object relations, self-psychology, and relational theories (Kohut, 1971; Mitchell, 1988; Winnicott, 1971). Given that psychotherapy was provided at a university-based outpatient clinic, treatments were often limited to 9 months. The mean treatment length was 26.61 sessions (*SD* = 6.47, range = 15–43).

Measures

Beck Depression Inventory-II (BDI-II; Beck, Steer, & Brown, 1996)

The BDI-II is a 21-item self-report measure of depression that asks respondents to rate the severity of their depressive symptoms during the previous two weeks using a Likert scale. Individual item scores are summed to create a total severity score with a range of 0–63 (Beck *et al.*, 1996). In the present sample (*N* = 18), the internal consistency of the BDI-II total score was good (α = .906).

Experiencing Scale (Klein et al.*, 1986)*

The Client Experiencing Scale is a 7-point ordinal measure that captures the degree to which clients orient to, symbolize, and use their internal felt experience as information to solve their problems. Regarding levels, EXP scale Level 1 expresses external and intellectual processing, where EXP Level 4 represents more internal contact with

experience as it is processed. The highest level, EXP Level 7, expresses fluid processing of presently occurring internal experiential referents. Raters use grammatical, expressive, and content distinctions to rate EXP levels. Previous studies have shown the EXP to have excellent reliability and validity (Pascual-Leone & Yeryomenko, 2017). Raters monitor the client's speech, and when they notice a change taking place vis-a-vis the EXP level, they assign a new score. At the end of the rating process, raters create two overall scores for each segment being analysed. The first is the most common score presented throughout the segment (modal), and the second is the highest score reached during that segment (peak). In all analyses, we reported only on the modal ratings, but the same pattern of results was shown when using the peak ratings.

Two-Person APES (HaCohen et al.*, 2017)*

The TPA[1] is an observer-rated coding system that examines the client's, and the therapist's, ability to make transitions between self-states. The TPA scale is coded for client and therapist concurrently. However, for the purpose of the current study, we were interested only in the coding conducted for the clients (Table S1 in the Supporting Information provides a detailed description of the TPA levels). As a dyadic tool, the TPA embodies the relational aspect of the therapist–client relationship at all levels of the scale. Thus, both client's and therapist's ratings embody relational aspects, even if treated or considered separately. The TPA levels present six possible relationships between self-states that are identified by three major rating elements: (1) the transition between the multiple self-states; (2) the client–therapist quality of relationship in the current interaction; and (3) emotional tone[2].

For the purpose of coding, each 50-min therapeutic session was divided into consecutive segments lasting 5 min. The raters listened to the recordings of the therapeutic sessions to provide a rating that best represented the quality of movement between self-states in each segment, evaluated based on the emotions and behaviours as they were expressed in the vocal and textual markers (see Table S1 in the Supporting Information). In this manner, each session received 10 rating scores, and the average score of the ratings was included in the analyses.

[1] *The development and validation of the TPA included several stages. During the first of these stages, a qualitative pilot research study was conducted producing several case studies (HaCohen* et al.*, 2018). Then, six key stages – which were based mostly on the APES levels but had relational aims and concerned the intersubjective influences – were conceptualized. In the next stage, 12 expert clinicians and theoreticians in the relational-psychodynamic field were asked to rate the TPA key stages, presented randomly, in terms of how they believed these stages reflected an optimal progression of the relational-dialectic approach in therapy regarding the patient and therapist. Their ratings indicated high agreement with the direction of development between levels. In the next stage, inter-rater reliability between two clinical judges for the rating of 48 therapeutic sessions yielded satisfactory results of intraclass correlation coefficient (ICC): Single measures = .91 for patient TPA and .94 for therapist TPA. Significant weak-to-moderate correlations were found between the TPA and the coherence coding system (Baerger & McAdams, 1999). In the final stage of development, the TPA extension was approved and supported by Professor William B. Stiles, the primary author of the original APES (Stiles, personal communication, June 2014) and by Professor Philip Bromberg, one of the founders of the relational-dialectic approach (Bromberg, personal communication, July 2014).*

[2] *TPA uses the* emotional tone *as a phenomenological marker of clients' ability to move between self-states. Unlike the EXP, it does not measure the ability to experience and process emotions. Thus, for example, while a sad tone may be included in a low-level TPA coding if the client is still avoiding another part of the self, it may be considered a high-level EXP coding if the client is actively experiencing and processing his/her sadness at that moment in therapy. On the other hand, a client's crying due to encountering two contradictory parts of the self will be rated as high on the TPA, but may be rated as a low-level EXP coding if the client is not expressing and processing his/her sadness.*

Raters

All raters were independent PhD and MA students in clinical psychology with two to five years of therapeutic experience. Different raters rated the EXP and the TPA. All raters completed the standard training (approximately 40 hr for the TPA and 60 hr for the EXP). At the time of the rating, the raters were masked to the treatment outcome. Inter-rater reliability between two clinical judges' ratings yielded satisfactory results (ICC for TPA = .91, ICC for EXP =.93).

Forming comparison groups and case selection

We used a contrasting groups design to maximize the difference between good- and poor-outcome cases. This method has been used in previous studies (e.g., Seggar, Lambert, & Hansen, 2002) and was used in the current research to divide the sample posteriori into two subgroups with regard to outcome. As most of the clients in our sample were diagnosed with affective disorders, we decided to use the reliable change index (RCI) of the BDI to form the comparison groups (Seggar *et al.*, 2002). The exclusion procedure resulted in 60 eligible cases from which 18 were selected through the formation of the comparison groups. The RCI of pre–post change on the BDI-II was computed for each eligible case, categorizing all clients into two subgroups: those whose change scores were above the RCI (n = 9) versus those who did not present any change in BDI scores, or whose scores even deteriorated (n = 9). The BDI scores of the good-outcome group dropped by 13.54 points on average, from between 17 and 40 at screening assessment (M = 24.5) to 2 and 21 post-treatment (M = 10). The BDI scores of the poor-outcome group increased by 6 points on average, ranging from 17 to 40 at screening (M = 25) and from 17 to 45 at post-treatment (M = 27). The groups also displayed significant differences in the change score from pre- to post-therapy, on the Outcome Questionnaire Self-Report (OQ-45.2; Lambert et al., 1996) – (OQ-45; M_{poor} = 7.0, SD_{poor} = 14.7; M_{good}, = −14.6, SD_{good} = 14.2 ($t(16) = -3.15$, p =.006), and on the Inventory of Interpersonal Problems Personality Questionnaire (IIP; Horowitz, 1999) (IIP; M_{poor} = −0.3, SD_{poor} = 0.6; M_{good}, = −1.0, SD_{good} = 0.6 ($t(16) = -2.44$, p = .025).

Session selection

For every dyad, five sessions were chosen in even intervals representing five phases in treatment. However, the first session was excluded as it might have had the quality of an initial interview, and the last session was excluded because its focus was presumed to be treatment termination (Crits-Christoph, Gibbons, Temes, Elkin, & Gallop, 2010). The intervals between the sessions were proportionate to the length of the entire treatment and varied from 2 to 3 sessions in the shortest treatment (15 sessions in length) to 9–10 sessions in the longest treatment (40 sessions in length). The average interval was spacing of five sessions. On occasions where we were undecided about which sessions should be sampled, we chose those that had complete data in terms of questionnaires and those that had higher audio-recording quality.

Procedure

The study was conducted in a university-based outpatient clinic between November 2013 and August 2015. The study procedures were part of the routine battery offered in the clinic. Clients and therapists consented to participate in the voluntary study, and they

were told that they could choose to terminate their participation in the study at any time with no effect on their treatment and that their anonymity would be preserved. The BDI was administered to clients as part of the intake procedure (i.e., at pre-treatment) and again at termination. All 90 sessions were audiotaped, transcribed, and then rated, using the TPA and the EXP scales.

Data analysis strategy

The data set had a hierarchical structure (sessions nested within clients); as a result, individual observations were not independent of one another. For this reason, to test the study's hypotheses, we used multilevel modeling (MLM; Hox, 2010). Specifically, to analyse the present data set, a two-level MLM was used, partitioning the total variability in TPA ratings for session *s* of client *c* into two components: within-client variability at Level 1, and between-client variability at Level 2. The variability of the predictor (i.e., EXP) was also partitioned into within-client and between-client levels. The former allowed testing of the first hypothesis regarding the association between TPA and EXP at the within-client level (i.e., whether in sessions in which EXP was higher, relative to the client's own average level, TPA was also higher), and the latter allowed testing of the association at the between-client level (i.e., whether clients with higher average levels of EXP, relative to the entire sample, also had higher TPA average levels). At Level 1, the predictor was created by centring the EXP score from each session for each client on the client's own mean (person-mean centring); at Level 2, the predictor was created by averaging the EXP scores from all sessions for each client, and then centring these person-mean scores on the sample's mean (grand-mean centring).

To test whether group moderated the associations between EXP and TPA (second hypothesis), we tested the interaction with Group (effect coded; Group = 0.5 for good-outcome group, Group = −0.5 for poor-outcome group) at both the within-client and between-client levels. Finally, to control for the linear effect of time, we entered Time (centred around the middle session, which was the 3rd coded session) as a covariate in the model (Bolger & Laurenceau, 2013; Curran & Bauer, 2011; Wang & Maxwell, 2015).

Thus, the mixed multilevel equation, in which both the intercept and the Level-1 predictor were considered to be random, was:

$$\begin{aligned}\mathrm{TPA}_{\mathrm{tc}} = {} & (\gamma_{00} + \gamma_{01} * \mathit{Group}_c + \gamma_{02} * \mathrm{Avg.\ EXP}_c \\ & + \gamma_{03} * \mathrm{Avg.\ EXP}_c * \mathrm{Group}_c + u_{0c}) + (\gamma_{10} + \gamma_{11} * \mathrm{Group}_c + u_{1c}) * \mathrm{EXP}_{\mathrm{ct}} \\ & + (\gamma_{20} + u_{2c}) * \mathrm{Time}_{\mathrm{tc}} + e_{\mathrm{tc}};\end{aligned}$$

where the TPA level at time *t* of client *c* was predicted by the sample's intercept (i.e., fixed effect; γ_{00}), the client's group deviation from the intercept (i.e., γ_{01}), this client's average level of EXP (i.e., γ_{02}), the interaction between this client's group and this client's average level of EXP (i.e., γ_{03}), and this client's deviation from the predicted intercept by all four of these fixed effects (i.e., random effect; u_{0c}). Additionally, the TPA level was predicted by the client's EXP level in this session multiplied by the sample's effects of within-client EXP (i.e., γ_{10}), the client's group deviation from these effects (i.e., γ_{11}), and this client's deviations from his/group's effect (i.e., u_{1c}); the TPA was also predicted by the time of this session multiplied by the sample's effect of time (i.e., γ_{20}) and this client's deviation from the sample's effect of time (i.e., u_{2c}). Finally, a Level-1 residual (e_{ct}) was included in the

equation to quantify for this session's deviation from all the previous effects. This Level-1 residual was allowed to differ from session to session, and its first-order autoregressive structure was estimated. Effect sizes were estimated with a semi-partial R^2 for linear mixed models (Edwards, Muller, Wolfinger, Qaqish, & Schabenberger, 2008).

Results

Preliminary analyses

Table 1 presents the descriptive statistics, along the zero-order correlations of the study's variables. As the table shows, both the EXP and TPA levels were higher in the good-outcome group than in the poor-outcome group. However, whereas the group differences in the TPA levels reached statistical significance, $t(88) = -2.44$, $p = .02$, the group differences in the EXP levels did not, $t(88) = -1.32$, $p = .19$. In addition, whereas the association between these variables was positive and significant for the good-outcome group, it was non-significant for the poor-outcome group.

Main analyses

The results of the fixed effects part of this model are presented in Table 2. As the table shows, and contrary to the first prediction, no association was found between EXP levels and TPA levels at either the between-client or the within-client levels. In partial support of the second prediction, whereas no interaction was found between EXP and group at the within-client level, the analysis yielded a significant interaction between EXP and group at the between-client level. As Figure 1 shows, and as was indicated by the simple effects analyses, whereas for clients in the good-outcome group, the association between average levels of EXP and TPA was positive and significant (Estimate = 0.40, $SE = .15$, $p = .016$), for clients in the poor-outcome group, this association was negative and significant (Estimate = −1.09, $SE = .38$, $p = .012$).

Discussion

The current research examined the association between two psychotherapy processes, which are viewed as central according to contemporary relational models, in good- and poor-outcome cases. Specifically, we explored the changes in the clients' quality of movement between self-states as well as changes in emotional experiencing, throughout psychotherapy. Although emotional experiencing has been rigorously studied and repeatedly correlated with psychotherapy success (e.g., Pos, Greenberg, & Warwar, 2009; Pascual-Leone & Yeryomenko, 2017), changes in the quality of movement between

Table 1. Descriptive statistics of EXP scores and TPA scores by outcome group

	Good-outcome group		Poor-outcome group	
	Mean (*SD*)	*r*	Mean (*SD*)	*r*
EXP	2.70 (0.96)	.507 ($p < .001$)	2.47 (0.70)	−.164 ($p = .283$)
TPA	2.14 (0.58)		1.82 (0.67)	

Note. EXP = Experiencing Scale; TPA = Two-Person APES.

Table 2. Fixed effects analysis – TPA scores predicted by EXP scores, comparison group, and the interaction

	Estimate (*SE*)	*T*	DF	*p*	Effect size
Intercept	1.91 (.09)	22.76	14	<.001	
Group	0.40 (.17)	2.42	14	.030	.29
EXP	−0.11 (.12)	−0.85	69	>.250	.01
Group * EXP	−0.01 (.24)	−0.06	69	>.250	.00
Avg. EXP	−0.34 (.20)	−1.69	14	.11	.17
Group * Avg. EXP	1.50 (.40)	3.7	14	.002	.49
Time	0.03 (.06)	0.47	69	>.250	.00

Note. EXP = Experiencing Scale; TPA = Two-Person APES.
Effect sizes were estimated with semi-partial R^2 for linear mixed models (Edwards et al., 2008), where $R^2 \leq .01$ represents small effect size; medium ≤ 0.09; and large ≤ 0.25.

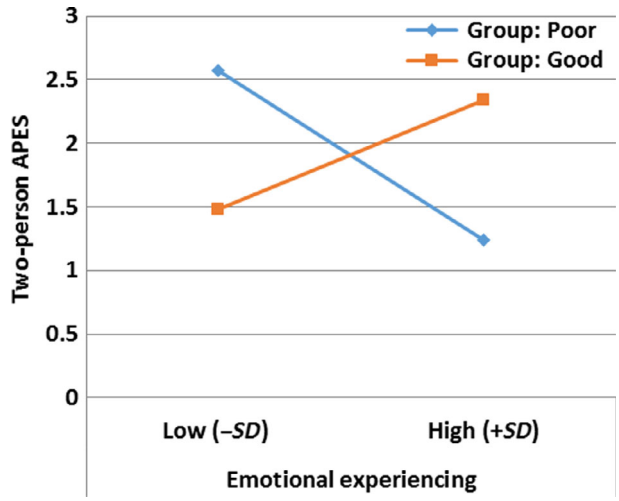


Figure 1. The EXP and TPA levels in the good- and poor-outcome groups. EXP = Experiencing Scale; TPA = Two-Person APES.

self-states, as measured by the TPA, form a new research focus which reflects the relational paradigmatic shift that views the self as comprising multiplicity (HaCohen *et al.*, 2017).

In the preliminary analyses, it was found that despite the fact that both emotional experiencing and self-states quality of movement (TPA) were higher in the good-outcome cases than in the poor-outcome cases, the difference between the groups reached statistical significance only for the TPA. The non-significant differences between the groups in emotional experiencing can be explained by the small sample size in the study. Yet, detecting statistical significance despite the small sample underlines the magnitude and consistency of the effects. Our results thus converge with previous studies linking improvement in the ability to tolerate the tension between multiple self-states with symptomatic relief (e.g., Basto *et al.*, 2018; Stiles, 2001). Nonetheless, these findings suggest that both processes are beneficial and have the potential to bring about therapeutic improvement.

Our hypotheses were that an association between TPA and emotional experiencing levels would be found and that this association would be more pronounced for clients presenting good outcomes. Analysing five sessions for each client allowed us to test this hypothesis both at the within-client level (i.e., testing whether sessions characterized by greater levels of TPA were also characterized by higher emotional experiencing) and at the between-client level (i.e., testing whether clients who presented higher TPA levels along treatment also presented greater emotional experiencing). Contrary to our first prediction, no association was found between self-states movement and emotional experiencing. These results can be understood in light of the results of the second prediction. In partial support of the second prediction, results revealed a significant interaction at the treatment level, showing that clients in the good-outcome group who on average exhibited higher levels of self-states movement (TPA levels) also exhibited higher levels of emotional experiencing on average. By contrast, clients in the poor-outcome group who on average exhibited higher levels in their quality of movement between self-states exhibited lower levels of emotional experiencing on average, and vice versa. At the session level, this interaction effect was not found.

The between-client level results support the argument that emotional experiencing within the realm of psychotherapeutic work may be substantially augmented by the capacity for flexible psychological movement and internal shifts (Pascual-Leone, 2009). In other words, it is important for one's emotional experiencing to encompass a wide range of multiple self-states, and not remain fixed upon a single stagnant one. In the same manner, when expanding one's capacity to dialectically acknowledge and accept their multiple self-states, an emotional component and connection should be incorporated therapeutically as well (Bucci, 2002). This growing ability of the client to make these psychological transitions can also reflect clients being less defensive and more open to being in touch with their inner worlds, which is one of the main goals of psychodynamic therapy (Winnicott, 1971).

The negative between-client correlation exhibited in the current research between self-states movement and emotional experiencing within the poor-outcome group may typify two client profiles. The first profile corresponds with clients who have high levels of emotional experiencing but lack psychological movement between multiple self-states. According to Bromberg (2016), when clients are able to experience deep emotions, but are only in contact with limited aspects of themselves, then threatening dissociated self-states remain cut-off and the experience of the self remains reduced. An extreme clinical example of this first profile is borderline personality disorder (BPD). Clients suffering from BPD are considered to possess emotional hyper-reactivity (Sansone & Sansone, 2010). Their interactions with others are emotionally intense and experienced through the prism of emotion, but they remain trapped in a narrow understanding of themselves and others. The second profile corresponds with clients who are able to *speak* about their conflict, but are unable to bear it emotionally. Meaning, they conceptualize the complex inner workings of their minds, but have no emotional connection to the implications of what is being expressed. A possible clinical example of this second profile is the defence mechanism of intellectualization. In this case, reasoning is used to block the confrontation with emotional stress – 'thinking is used to avoid feeling' (Gabbard, 2010, p. 35). These two profiles clarify how self-states dialectics and meaningful emotional experiencing are two psychological mechanisms in psychotherapy that complement one another and are mutually required for the achievement of a positive outcome.

Taken together, these findings suggest a more complex view of the contribution of clients' ability to move between self-states and emotional experiencing and processing.

That is, EXP and TPA jointly predict treatment outcome in a different way than does each measure individually. These findings may suggest that clinicians and researchers should be attentive to the co-occurrence of related processes during psychotherapy. Most previous studies have examined the association of a single therapeutic process with treatment outcome (for review, see Crits-Christoph, Gibbons, & Mukherjee, 2013). Studies examining how different processes interact to predict treatment outcome have revealed interesting results. Rubel, Rosenbaum, and Lutz (2017), for example, found a significant interaction effect between clients' emotional experiencing and clients' coping ability in predicting treatment outcome. Specifically, clients who reported a higher coping ability showed greater symptomatic improvement if they reported greater emotional experiencing as well. However, for clients with low emotional experiencing, coping ability no longer predicted symptomatic change. This finding may also suggest that emotional experiencing must accompany other therapeutic processes. To illustrate, we can think of emotional experiencing as the fuel that propels the car, and the movement between self-states as the steering wheel that keeps the car on course. Without the presence of both elements, the car either will not move at all or will head in the wrong direction.

The non-significant results on the within-client level may suggest that the two processes may mutually improve over the course of therapy but not necessarily within the same session; that is, the two processes may follow each other in a circular fashion rather than co-occurring, resulting in corresponding average levels but not session levels. Relatedly, it is possible that growth in these two processes occurs at different times in therapy. This explanation aligns with the different patterns of change found in previous studies for the EXP and TPA. While previous studies revealed a linear increase in EXP over the course of therapy (Whelton, 2004), a curvilinear change (with an increase at the beginning of therapy and a decrease later in therapy) was indicated for the TPA (HaCohen *et al.*, 2018). Future studies analysing several sequential sessions from each client may be able to test the within-client evolution of these two processes.

Several limitations of the current study should be noted. First, this study was designed as a naturalistic field exploration. Although the internal validity of such a design is more limited, it has an advantage in terms of external validity as it more accurately reflects the reality of clinical work with clients in public clinics (Ablon, Levy, & Katzenstein, 2006; Bond & Perry, 2004). Second, although all therapists received intensive supervision, their relatively minimal experience may limit our ability to generalize the results to more experienced clinicians.

An additional study limitation was its reliance on correlational data, which precludes any strong inference of causal associations between self-states movement, emotional experiencing patterns, and treatment outcomes. The possibility of reverse causation (e.g., the idea that achieving symptomatic relief might affect movement between self-states as well as emotional experiencing), and the possibility that some unmeasured variable could have influenced all three constructs, must be considered.

Furthermore, the relative newness of the TPA measure is an additional limitation. The development of this measure was carried out by an extensive team of experts in order to establish the tool's initial merits and empirical utility. Still, more research is needed to further strengthen its validity and reliability (for extended information regarding the development, reliability, and validity of the TPA, see HaCohen *et al.*, 2017). Finally, although the BDI is a widely validated measure and was used in previous studies for assessing comparison groups (e.g., Detert, Llewelyn, Hardy, Barkham, & Stiles, 2006), additional measures may strengthen and enrich the assessment of what is considered

good- and *poor*-outcome cases. Relatedly, the lack of assessment of personality disorders in the sample may also limit the generalizability of the findings. As noted earlier, specific personality disorders (i.e., over-controlled vs. dysregulated) may affect the way the TPA and EXP interact to predict treatment outcome.

Data availability statement

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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Supporting Information

The following supporting information may be found in the online edition of the article:

Table S1. Two-person APES scale (TPA) with clinical explication.

View publication stats